

OBSTETRICS NURSING 1 QUESTIONS

Question 1

A woman who is fully dilated begins pushing during labor. Which major physiological event occurs during the second stage of labor?

- a. Cervical dilation
- b. Expulsion of the fetus
- c. Separation of the placenta
- d. Cervical effacement

☒ Correct Answer: b. Expulsion of the fetus

Rationale: The second stage of labor begins with full cervical dilation and ends with the birth of the baby.

Question 2

A prolonged second stage of labor leads the provider to perform an episiotomy. What is the main purpose of this procedure?

- a. To reduce maternal exhaustion
- b. To prevent fetal distress
- c. To shorten the third stage of labor
- d. To enlarge the vaginal opening for delivery

☒ Correct Answer: d. To enlarge the vaginal opening for delivery

Rationale: An episiotomy widens the vaginal outlet to facilitate delivery when necessary.

Question 3

After delivery of the placenta, the nurse performs fundal massage. What is the primary reason for this action?

- a. To assist placental expulsion
- b. To relieve maternal discomfort
- c. To aid perineal repair
- d. To promote uterine contraction and prevent hemorrhage

☒ Correct Answer: d. To promote uterine contraction and prevent hemorrhage

Rationale: Fundal massage stimulates uterine contraction, reducing the risk of postpartum hemorrhage.

Question 4

A pregnant woman experiences irregular contractions that subside with rest. How should the nurse explain Braxton Hicks contractions?

- a. They are regular and progressively stronger
- b. They increase in duration over time
- c. They signify the onset of true labor
- d. They decrease with rest and hydration

☒ Correct Answer: d. They decrease with rest and hydration

Rationale: Braxton Hicks contractions are false labor pains and subside with rest.

Question 5

A newborn has difficulty latching during the first breastfeeding attempt. What is the most appropriate nursing action?

- a. Delay breastfeeding until later
- b. Supplement with formula
- c. Encourage the mother to continue without help
- d. Assist the mother to achieve proper latch

☒ Correct Answer: d. Assist the mother to achieve proper latch

Rationale: Hands-on guidance promotes effective breastfeeding and prevents nipple trauma.

Question 6

Late decelerations are observed on the fetal heart monitor during the second stage of labor. What should the nurse do first?

- a. Reposition the mother and administer oxygen

- b. Encourage stronger pushing
- c. Prepare for emergency cesarean section
- d. Continue observation only

 Correct Answer: a. Reposition the mother and administer oxygen

Rationale: Late decelerations indicate uteroplacental insufficiency; improving oxygenation is the priority.

Question 7

A woman reports severe pain during labor despite an epidural. What is the nurse's best response?

- a. Reassess epidural effectiveness and reposition the patient
- b. Give additional analgesics immediately
- c. Encourage stronger pushing
- d. Tell her the pain will soon end

 Correct Answer: a. Reassess epidural effectiveness and reposition the patient

Rationale: Epidurals may not provide complete relief and require reassessment.

Question 8

A woman with a history of postpartum hemorrhage is in the third stage of labor. Which medication is most appropriate to prevent hemorrhage?

- a. Oxytocin
- b. Nifedipine and oxytocin
- c. Misoprostol
- d. Nifedipine

 Correct Answer: a. Oxytocin

Rationale: Oxytocin promotes uterine contraction and is the first-line drug for preventing postpartum hemorrhage.

Question 9

Severe lower back pain during labor suggests which fetal position?

- a. Occiput posterior
- b. Occiput anterior
- c. Breech
- d. Transverse lie

☒ Correct Answer: a. Occiput posterior

Rationale: The fetal occiput presses against the maternal sacrum, causing back pain.

Question 10

During the second stage of labor, what is the function of the perineum?

- a. Protects the fetal head
- b. Supports uterine contractions
- c. Regulates contraction strength
- d. Stretches to allow passage of the fetal head

☒ Correct Answer: d. Stretches to allow passage of the fetal head

Rationale: The perineum expands to accommodate the descending fetal head.

Question 11

A fetal heart rate deceleration is noted during labor. What is the nurse's first action?

- a. Administer oxygen
- b. Reposition the mother
- c. Call the physician
- d. Increase IV fluids

☒ Correct Answer: b. Reposition the mother

Rationale: Maternal repositioning improves uteroplacental blood flow.

Question 12

A postpartum woman has moderate lochia rubra. What should the nurse do?

- a. Inform the doctor immediately
- b. Increase IV fluids
- c. Document as a normal finding
- d. Assess for infection

☒ Correct Answer: c. Document as a normal finding

Rationale: Moderate lochia rubra is normal in the early postpartum period.

Question 13

Which finding indicates full cervical dilation?

- a. Cervix dilated 5 cm
- b. Cervix dilated 10 cm and fully effaced
- c. Cervix dilated 8 cm
- d. Cervix partially effaced

☒ Correct Answer: b. Cervix dilated 10 cm and fully effaced

Rationale: Full dilation (10 cm) allows fetal passage.

Question 14

Which vital sign is most concerning in a woman in labor?

- a. Temperature 38.6°C
- b. Respiratory rate 24/min
- c. BP 120/80 mmHg
- d. Pulse 90 bpm

☒ Correct Answer: a. Temperature 38.6°C

Rationale: Fever may indicate infection and requires prompt evaluation.

Question 15

A woman at +3 station reports strong rectal pressure. What should the nurse do?

- a. Reassess fetal heart rate

- b. Increase IV fluids
- c. Prepare for imminent delivery
- d. Stop the patient from pushing

☒ Correct Answer: c. Prepare for imminent delivery

Rationale: +3 station indicates the fetus is very low in the birth canal.

Question 16

Sudden bright red bleeding occurs after placental delivery. This most likely indicates:

- a. Cervical laceration
- b. Uterine rupture
- c. Retained placenta
- d. Normal bleeding

☒ Correct Answer: a. Cervical laceration

Rationale: Bright red bleeding with a firm uterus suggests trauma.

Question 17

What does “crowning” mean during labor?

- a. Fetal head passing cervix
- b. Fetal descent into pelvis
- c. Delivery of shoulders
- d. Fetal head visible at vaginal opening

☒ Correct Answer: d. Fetal head visible at vaginal opening

Rationale: Crowning occurs when the head remains visible between contractions.

Question 18

A blood-tinged mucous discharge before labor is called:

- a. Lightening
- b. Rupture of membranes

c. Engagement

d. Bloody show

☒ Correct Answer: d. Bloody show

Rationale: Bloody show results from cervical dilation and mucus plug release.

Question 19

A fetal heart rate drop to 90 bpm lasting 2 minutes is termed:

a. Late deceleration

b. Prolonged deceleration

c. Variable deceleration

d. Early deceleration

☒ Correct Answer: b. Prolonged deceleration

Rationale: Prolonged decelerations last more than 2 minutes but less than 10.

Question 20

Which muscles are primarily used during pushing in the second stage of labor?

a. Arm and chest muscles

b. Uterine and cervical muscles

c. Abdominal and pelvic floor muscles

d. Back and leg muscles

☒ Correct Answer: c. Abdominal and pelvic floor muscles

Rationale: These muscles assist voluntary pushing.

Question 21

What is the best nursing action when a mother reports difficulty with breastfeeding latch?

a. Introduce formula

b. Demonstrate breastfeeding positions

c. Delay feeding

d. Reassure only

☒ Correct Answer: b. Demonstrate breastfeeding positions

Rationale: Proper positioning improves latch success.

Question 22

To prevent postpartum hemorrhage, the nurse should:

a. Massage fundus and give oxytocin

b. Place patient in Trendelenburg position

c. Monitor BP every 30 minutes

d. Encourage ambulation

☒ Correct Answer: a. Massage fundus and give oxytocin

Rationale: These measures promote uterine contraction.

Question 23

Variable decelerations during labor should first be managed by:

a. Repositioning the mother

b. Emergency cesarean section

c. Increasing IV fluids

d. Administering oxygen

☒ Correct Answer: a. Repositioning the mother

Rationale: Variable decelerations are usually due to cord compression.

Question 24

An effective way to reduce back pain in occiput posterior labor is:

a. Apply counterpressure

b. Place patient supine

c. Increase IV fluids

d. Give muscle relaxants

☒ Correct Answer: a. Apply counterpressure

Rationale: Counterpressure relieves sacral pressure.

Question 25

The recommended management for a fetus in transverse lie is:

a. Vaginal delivery

b. Allow labor to continue

c. Cesarean section

d. Immediate external version

☒ Correct Answer: c. Cesarean section

Rationale: Vaginal delivery is not possible in transverse lie.

Question 26

An anxious woman in active labor should be managed by:

a. Continuous monitoring

b. Breathing and relaxation techniques

c. Sedatives

d. IV fluids

☒ Correct Answer: b. Breathing and relaxation techniques

Rationale: Non-pharmacologic methods reduce anxiety safely.

Question 27

A postpartum woman voids only 100 mL in 2 hours. What should the nurse do next?

a. Give diuretics

b. Encourage fluids

c. Check for bladder distension

d. Insert catheter

☒ Correct Answer: c. Check for bladder distension

Rationale: A full bladder can cause urinary retention and uterine atony.

Question 28

A pad soaked in 30 minutes with bright red blood indicates:

- a. Give oxytocin
- b. Notify healthcare provider
- c. Reassure patient
- d. Document only

☒ Correct Answer: b. Notify healthcare provider

Rationale: This suggests postpartum hemorrhage.

Question 29

A fetal station of 0 means:

- a. Head at level of ischial spines
- b. Head not engaged
- c. Head floating
- d. Head crowning

☒ Correct Answer: a. Head at level of ischial spines

Rationale: The ischial spines mark 0 station.

Question 30

Early decelerations during pushing that recover quickly require the nurse to:

- a. Continue monitoring
- b. Reposition patient
- c. Prepare for surgery
- d. Stop pushing

☒ Correct Answer: a. Continue monitoring

Rationale: Early decelerations are normal and caused by head compression.